- **6760 Jimmy Carter Blvd Suite 165 Norcross, GA 30071. Ph: (678) 969-0904 Fax: (678) 969-0908**
- **6568 Tara Blvd. Suite B Jonesboro, GA 30236. Ph: (678) 545-6778 Fax: (678) 489-2981**
- **955-E Interstate Ridge Business Park, Gainesville, GA 30501. Ph: (678) 989-4566 Fax: (678) 989-4575**

PATIENT NAME:	Merriman, Raquel Katoria	**MR#:**	MERR174779
DATE OF EXAM	22-Mar-2024 11:24 AM	**DOB:**	06-Dec-1994
EXAM REQUESTED:	MRI CERVICAL W/O		
REFERRING PHYSICIAN:	ANGELA ALVAREZ		

HISTORY: Neck pain due to motor vehicle accident. Date of accident 02/13/2024

TECHNIQUE: The images were obtained in the sagittal and axial plane without contrast.

FINDINGS: Signal intensity from the cervical vertebrae is normal. There is a normal degree of lordosis. There is no evidence of fracture.

Signal intensity from the cord is normal without evidence of contusion, syrinx or myelomalacia. The cerebellar tonsils and fourth ventricle are in the normal position.

The C2-3 disc space level shows central herniated disc 1 x 3 mm

The C3-4 disc space level shows the right paracentral herniated disc .5 x 3 mm

The C4-5 disc space level shows no evidence of herniated disc, spinal stenosis or neural foraminal impingement.

The C5-6 disc space level shows no evidence of herniated disc, spinal stenosis or neural foraminal impingement.

The C6-7 disc space level shows no evidence of herniated disc, spinal stenosis or neural foraminal impingement. Bulging disc in the midline.

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PATIENT NAME:	Merriman, Raquel Katoria	**MR#:**	MERR174779
DATE OF EXAM	22-Mar-2024 11:24 AM	**DOB:**	06-Dec-1994
EXAM REQUESTED:	MRI CERVICAL W/O		
REFERRING PHYSICIAN:	ANGELA ALVAREZ		

The C7-T1 disc space level shows no evidence of herniated disc, spinal stenosis or neural foraminal impingement.

IMPRESSION: Herniated discs C2-3 and C3-4

Please note that annular tears are suggestive of an active inflammatory process, which may have been propagated and/or aggravated by the reported injury and has been implicated in creating radicular type symptomatology and/or focal pain. Clinical correlation for corresponding posttraumatic pain and radicular involvement is advised.

THIS REPORT WAS ELECTRONICALLY SIGNED

Dr. Thomas Turek
Board Certified American College of Radiology

Thomas E. Turek, MD.

Electronically signed:
Turek, Thomas MD 25-Mar-2024 11:46 AM
d/t
Dictated by: Turek, Thomas MD

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- **955-E Interstate Ridge Business Park, Gainesville, GA 30501. Ph: (678) 989-4566 Fax: (678) 989-4575**

PATIENT NAME:	Merriman, Raquel Katoria	**MR#:**	MERR174779
DATE OF EXAM	22-Mar-2024 11:35 AM	**DOB:**	06-Dec-1994
EXAM REQUESTED:	MRI LEFT WRIST W/O		
REFERRING PHYSICIAN:	ANGELA ALVAREZ		

MRI OF THE LEFT WRIST

HISTORY: Wrist pain due to motor vehicle accdent. Date of accident 02/13/2024.

TECHNIQUE: Images were obtained in the sagittal, axial and coronal plane.

FINDINGS: The signal intensity from the bony structures appears normal. There is no evidence of fracture, bone destruction or avascular necrosis. There is no evidence of radiocarpal arthropathy or intercarpal arthropathy.

The TFC demonstrates a small intrasubstance tear at the attachment point of the ulnar styloid. . The scapholunate ligament and triquetral lunate ligaments are also intact. There is no evidence of carpal instability.

The flexor and extensor tendons are normal. The median nerve is normal. There is no evidence of ganglion cyst.

There is no evidence of negative or positive ulnar variance. The region of Guyon's canal appears normal without mass effect.

IMPRESSION: Small intrasubstance tear of the TFC attachment point to the ulner styloid.

THIS REPORT WAS ELECTRONICALLY SIGNED

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- **6568 Tara Blvd. Suite B Jonesboro, GA 30236. Ph: (678) 545-6778 Fax: (678) 489-2981**
- **955-E Interstate Ridge Business Park, Gainesville, GA 30501. Ph: (678) 989-4566 Fax: (678) 989-4575**

PATIENT NAME:	Merriman, Raquel Katoria	**MR#:**	MERR174779
DATE OF EXAM	22-Mar-2024 11:35 AM	**DOB:**	06-Dec-1994
EXAM REQUESTED:	MRI LEFT WRIST W/O		
REFERRING PHYSICIAN:	ANGELA ALVAREZ		

Dr. Thomas Turek
Board Certified American College of Radiology

Thomas E. Turek, MD

Electronically signed:
Turek, Thomas MD 25-Mar-2024 11:45 AM
d/t
Dictated by: Turek, Thomas MD

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Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

Initial Exam

Patient Name: Raquel Merriman
Date of Birth: 12/6/1994

Provider: Meka Kennedy, DC
Date of Service: 3/2/2024

SUBJECTIVE

HISTORY OF PRESENT ILLNESS

She has never experienced this condition before.

PRIMARY COMPLAINT

Left neck, Right neck

- Aching, Throbbing pain: 9/10
  Radiates to (posterior) left shoulder, left arm, left elbow, left wrist, left hand and left finger. Described as constant (up to 100% of day) of severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Pain with motion: 10/10
  Described as constant (up to 100% of day) of severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Stiffness: 3/10
  Described as intermittent (up to 50% of day).

SECOND COMPLAINT

Left shoulder

- Stabbing, Sharp, Aching pain: 9/10
  Described as constant (up to 100% of day) of severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.
- Pain with motion: 10/10
  Described as constant (up to 100% of day) of severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.

THIRD COMPLAINT

Left finger, Left hand, Left wrist

- Aching, Sharp pain: 9/10
  Described as constant (up to 100% of day) of severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by talking on phone, working, driving, changing positions and ADL's.

- Pain with motion: 10/10
  Described as constant (up to 100% of day) of severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by talking on phone, working, driving, changing positions and ADL's.

FOURTH COMPLAINT
Left upper back, Right upper back

- Pain: 7/10
  Described as intermittent (up to 50% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, working, driving, changing positions and ADL's.

FIFTH COMPLAINT
Head

- Aching, Throbbing pain: 7/10
  Radiates to (posterior) head. Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by sleeping and ADL's.

PAST, FAMILY, SOCIAL HISTORY
Surgeries/Hospitalizations: Carpal Tunnel Surgery
Allergies: None
Medications: None
Prior conditions: None
Collision occurred on 2/13/2024 at approximately 2:30 PM.
The patient was travelling straight ahead while another driver attempted a left turn , resulting in the collision
Symptoms reported: Patient is facing sleep disturbances, headache & intermittent pain in lower part of their neck following an accident

Mrs. Merriman was the driver of the vehicle, at the time of the collision. There was passenger side damage to the vehicle. The collision happened while she was slowly moving at Kelly Road, GA. Street surface was dry. The impact on the vehicle was at the rear end. Upon impact, the brakes were locked, and the car did not move. Mrs. Merriman was seated in the vehicle as a driver and was wearing a seat belt. The vehicle is equipped with air bags and they did not deploy. Mrs. Merriman didn't see the impact coming. Upon impact, her head was looking ahead and she was thrown forward. Mrs. Merriman remembers the collision happening.
She was in
* X-rays were taken of the neck
* Medications given: Muscle relaxants .

She reports that she went to the Piedmont Henry Hospital on 2/14/2024 for further evaluation. She reports that x-rays of her neck and left shoulder was taken. She was prescribed Tizanidine (muscle relaxer) and Diclofenac for inflammation and pain. She states that she took her medication for two days and she did not get any relief.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; C5; C6; T1; T3; T5.

VITALS

Pulse rate: 61 BPM (relaxed)

Blood Pressure (seated): right -N/A left-105/66 mmHg

Tenderness

Left Side

- Cervical Spine (minimal)

Right Side

- Cervical Spine (minimal)

Spasm/Hypertonicity

Left Side

- Cervical Spine (moderate-severe)
- Shoulder (moderate-severe)

Right Side

- Cervical Spine (moderate-severe)

GAIT

Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
Foraminal compression test[1]	Positive	Positive	
Shoulder depression test[2]	Positive	WNL	
Spinal percussion test-cervical[3]	Positive	Positive	
Apley scratch test[4]	Positive	Positive	

[1]Radicular pain could indicate pressure on the nerve root. Localized pain could indicate sprain or strain or foraminal encroachment.

[2]Pain in the arm could indicate adhesions of the nerve roots. Local pain could indicate sprain or strain or adhesions of the shoulder joint capsule.

[3]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

[4]Increased pain indicates tendinitis of the supraspinatus tendon.

POSTURE

- High right shoulder

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	40° (80% of normal)	Pain, Spasm	
Extension	50° (83% of normal)	Pain, Spasm	
Left lateral flexion	30° (66% of normal)	Pain, Spasm, Guarding	
Right lateral flexion	25° (55% of normal)	Pain, Spasm, Guarding	
Left rotation	75° (93% of normal)	Pain, Spasm	

Right rotation within normal limits.
3/2/2024 She reports that she was diagnosed with carpal tunnel in her left hand around 2019 or 2020.
She reports that she works for Amazon 4 days out of the week for 8 to 12 hours a day depending on her work load. She states that she has to pick up packages and she has more pain grabbing the packages due to the swelling and injury to her left hand.

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Radiculopathy, cervical region (M54.12)
7. Cervicalgia (M54.2)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
11. Segmental and somatic dysfunction of thoracic region (M99.02)
12. Segmental and somatic dysfunction of upper extremity (M99.07)
13. Strain of muscle(s) and tendon(s) of the rotator cuff of left shoulder, initial encounter (S46.012A)
14. Unspecified superficial injury of left hand, initial encounter (S60.922A)
15. Pain in joints of left hand (M25.542)
16. Other muscle spasm (M62.838)
17. Sleep deprivation (Z72.820)

LONG-TERM GOALS

- Increase cervical range of motion.
- Decrease cervical pain .
- Decrease thoracic pain.
- Decrease shoulder pain.
- Decrease left shoulder pain.
- Be able to work pain free.
- Be able to change positions without increase in.
- Be able to drive without pain.

PLAN

Based on subjective and objective data, today's treatment consists of:

- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Left Scapula, Right Scapula, Left Rib Cage, Right Rib Cage, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Right Scapula, Left Scapula, Left Rib Cage, Right Rib Cage, Back, to

reduce pain reported at initial encounter by 25%.

- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, to reduce pain reported at initial encounter by 25%.
- New Patient evaluation and management service, 99204, was provided today.

Patient received care without incident.

(Electronic Signature)
Provider Name: Meka Kennedy, DC
Date: 03/05/2024 04:21 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Raquel Merriman
Date of Birth: 12/6/1994

Provider: Marcia Smith, DC
Date of Service: 3/7/2024

SUBJECTIVE

PRIMARY COMPLAINT
Left neck, Right neck

- Aching, Throbbing pain: 9/10
  Radiates to (posterior) left shoulder, left arm, left elbow, left wrist, left hand and left finger. Described as constant (up to 100% of day) of severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Pain with motion: 10/10
  Described as constant (up to 100% of day) of severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Stiffness: 3/10
  Described as intermittent (up to 50% of day).

SECOND COMPLAINT
Left shoulder

- Stabbing, Sharp, Aching pain: 9/10
  Described as constant (up to 100% of day) of severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.
- Pain with motion: 10/10
  Described as constant (up to 100% of day) of severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.

THIRD COMPLAINT
Left finger, Left hand, Left wrist

- Aching, Sharp pain: 9/10
  Described as constant (up to 100% of day) of severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by talking on phone, working, driving, changing positions and ADL's.
- Pain with motion: 10/10
  Described as constant (up to 100% of day) of severe intensity (symptom is not alleviated

by any amount of modification to ADL); increased by talking on phone, working, driving, changing positions and ADL's.

FOURTH COMPLAINT
Left upper back, Right upper back

- Pain: 7/10
  Described as intermittent (up to 50% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, working, driving, changing positions and ADL's.

FIFTH COMPLAINT
Head

- Aching, Throbbing pain: 7/10
  Radiates to (posterior) head. Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by sleeping and ADL's.

Patient was **dx'd with carpal tunnel 5 years ago**. **The pain** and **carpal tunnel was under control until the day of the accident which caused it to flare up**. **She is unable to grab things strongly**, She states **nothing helps the pain not even naproxen**. **The pain is shooting into** her **hand** and **up her arm**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C4; C5; C6; T1; T3; T5**.**

Tenderness

Left Side

- Cervical Spine (Minimal)

Right Side

- Cervical Spine (Minimal)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Radiculopathy, cervical region (M54.12)
7. Cervicalgia (M54.2)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)

10. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
11. Segmental and somatic dysfunction of thoracic region (M99.02)
12. Segmental and somatic dysfunction of upper extremity (M99.07)
13. Strain of muscle(s) and tendon(s) of the rotator cuff of left shoulder, initial encounter (S46.012A)
14. Unspecified superficial injury of left hand, initial encounter (S60.922A)
15. Pain in joints of left hand (M25.542)
16. Other muscle spasm (M62.838)
17. Sleep deprivation (Z72.820)

PLAN

Based on subjective and objective data, today's treatment consists of:

- **CMT 1-2 Regions (98940) consisting of activator technique, diversified technique was provided specifically to C1; C4; C5; C6; T1; T3; T5, to reduce pain reported at initial encounter by 25%.**
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Left Scapula, Right Scapula, Left Rib Cage, Right Rib Cage, Back, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Right Scapula, Left Scapula, Left Rib Cage, Right Rib Cage, Back, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marcia Smith, DC
Date: 03/07/2024 10:24 AM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Raquel Merriman
Date of Birth: 12/6/1994

Provider: Angela Alvarez, DC
Date of Service: 3/8/2024

SUBJECTIVE

PRIMARY COMPLAINT
Left neck, Right neck

- Aching, Throbbing pain: **From 9 to 8/10 (10% decrease)**
  Radiates to (posterior) left shoulder, left arm, left elbow, left wrist, left hand and left finger. Described as constant (up to 100% of day) of **moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL)**; increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Pain with motion: **From 10 to 9/10 (10% decrease)**
  Described as constant (up to 100% of day) of severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Stiffness: 3/10
  Described as intermittent (up to 50% of day).

SECOND COMPLAINT
Left shoulder

- Stabbing, Sharp, Aching pain: 9/10
  Described as constant (up to 100% of day) of severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.
- Pain with motion: **From 10 to 9/10 (10% decrease)**
  Described as constant (up to 100% of day) of severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.

THIRD COMPLAINT
Left finger, Left hand, Left wrist

- Aching, Sharp pain: 9/10
  Described as **frequent (up to 75% of day)** of **moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL)**; increased by talking on phone, working, driving, changing positions and ADL's.
- Pain with motion: **From 10 to 9/10 (10% decrease)**
  Described as **frequent (up to 75% of day)** of **moderate to severe intensity (symptom is**

not alleviated by any amount of modification to ADL); increased by talking on phone, working, driving, changing positions and ADL's.

FOURTH COMPLAINT
Left upper back, Right upper back

- Pain: 7/10
  Described as intermittent (up to 50% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, working, driving, changing positions and ADL's.

FIFTH COMPLAINT
Head

- Aching, Throbbing pain: 7/10
  Radiates to (posterior) head. Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by sleeping and ADL's.

3/8/24- Pt **reports she feels on the left side of her neck throbbing pounding** pain, and **when** she is **sitting when looking** in her **peripheral feels it immediately**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C4; C5; C6; T1; T5**. No findings: C1; T3**.

Tenderness

Left Side
- Cervical Spine (Minimal)

Right Side
- Cervical Spine (Minimal)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)

Right Side
- Cervical Spine (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Radiculopathy, cervical region (M54.12)
7. Cervicalgia (M54.2)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
11. Segmental and somatic dysfunction of thoracic region (M99.02)

12. Segmental and somatic dysfunction of upper extremity (M99.07)
13. Strain of muscle(s) and tendon(s) of the rotator cuff of left shoulder, initial encounter (S46.012A)
14. Unspecified superficial injury of left hand, initial encounter (S60.922A)
15. Pain in joints of left hand (M25.542)
16. Other muscle spasm (M62.838)
17. Sleep deprivation (Z72.820)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of activator technique, diversified technique was provided specifically to C4; C5; C6; T1; T5, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, **left back**, **left shoulder**, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to **left neck**, **right neck**, **left back**, **right** back, **left shoulder**, **left hand**, to reduce pain reported at initial encounter by 25%.
- **Therapeutic Exercise (Ea. 15 Min) (97110) consisting of cervical active therapeutic movements, lumbo-pelvic knee to chest stretch was provided specifically to Neck, Back for 8 minutes (1 unit), to reduce pain reported at initial encounter by 25%.**

Patient received care without incident.

(Electronic Signature)
Provider Name: Angela Alvarez, DC
Date: 03/08/2024 10:39 AM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Raquel Merriman
Date of Birth: 12/6/1994

Provider: Angela Alvarez, DC
Date of Service: 3/9/2024

SUBJECTIVE

PRIMARY COMPLAINT
Left neck, Right neck

- Aching, Throbbing pain: 8/10
  Radiates to (posterior) left shoulder, left arm, left elbow, left wrist, left hand and left finger. Described as **frequent to constant (up to between 75% and 100% of day)** of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Pain with motion: **From 9 to 8/10 (10% decrease)**
  Described as **frequent to constant (up to between 75% and 100% of day)** of **moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL)**; increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Stiffness: 3/10
  Described as intermittent (up to 50% of day).

SECOND COMPLAINT
Left shoulder

- Stabbing, Sharp, Aching pain: **From 9 to 8/10 (10% decrease)**
  Described as **frequent to constant (up to between 75% and 100% of day)** of **moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL)**; increased by lifting, working, driving, changing positions and ADL's.
- Pain with motion: 9/10
  Described as **frequent to constant (up to between 75% and 100% of day)** of **moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL)**; increased by lifting, working, driving, changing positions and ADL's.

THIRD COMPLAINT
Left finger, Left hand, Left wrist

- Aching, Sharp pain: 9/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by talking on phone, working, driving, changing positions and ADL's.

- Pain with motion: 9/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by talking on phone, working, driving, changing positions and ADL's.

FOURTH COMPLAINT
Left upper back, Right upper back

- Pain: 7/10
  Described as intermittent (up to 50% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, working, driving, changing positions and ADL's.

FIFTH COMPLAINT
Head

- Aching, Throbbing pain: 7/10
  Radiates to (posterior) head. Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by sleeping and ADL's.

3/8/24- Pt reports she feels on the left side of her neck throbbing pounding pain, and when she is sitting when looking in her peripheral feels it immediately.

3/9/24- Pt reports she does not feel well today.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C4; C6; T1; T5**. No findings: C5**.

Tenderness

Left Side
- Cervical Spine (Minimal)

Right Side
- Cervical Spine (Minimal)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)

Right Side
- Cervical Spine (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Radiculopathy, cervical region (M54.12)
7. Cervicalgia (M54.2)

8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
11. Segmental and somatic dysfunction of thoracic region (M99.02)
12. Segmental and somatic dysfunction of upper extremity (M99.07)
13. Strain of muscle(s) and tendon(s) of the rotator cuff of left shoulder, initial encounter (S46.012A)
14. Unspecified superficial injury of left hand, initial encounter (S60.922A)
15. Pain in joints of left hand (M25.542)
16. Other muscle spasm (M62.838)
17. Sleep deprivation (Z72.820)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of activator technique, diversified technique was provided specifically to C4; C6; T1; T5, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, left back, left shoulder, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to left neck, right neck, left back, right back, left shoulder, left hand, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Angela Alvarez, DC
Date: 03/09/2024 09:56 AM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Raquel Merriman
Date of Birth: 12/6/1994

Provider: Marcia Smith, DC
Date of Service: 3/14/2024

SUBJECTIVE

PRIMARY COMPLAINT
Left neck, Right neck

- Aching, Throbbing pain: 8/10
  Radiates to (posterior) left shoulder, left arm, left elbow, left wrist, left hand and left finger. Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Pain with motion: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Stiffness: 3/10
  Described as intermittent (up to 50% of day).

SECOND COMPLAINT
Left shoulder

- Stabbing, Sharp, Aching pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.
- Pain with motion: 9/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.

THIRD COMPLAINT
Left finger, Left hand, Left wrist

- Aching, Sharp pain: 9/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by talking on phone, working, driving, changing positions and ADL's.

- Pain with motion: 9/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by talking on phone, working, driving, changing positions and ADL's.

FOURTH COMPLAINT
Left upper back, Right upper back

- Pain: 7/10
  Described as intermittent (up to 50% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, working, driving, changing positions and ADL's.

FIFTH COMPLAINT
Head

- Aching, Throbbing pain: 7/10
  Radiates to (posterior) head. Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by sleeping and ADL's.

Patient states she is having left arm pain that is shooting down her neck and into her shoulder and jaw. **Pt also reports chest pain for the last few days**. **Blood pressure is stable**. **Headaches have not subsided.**
Reports hand weakness is getting worse and grip strength is decreasing.

3/8/24- Pt reports she feels on the left side of her neck throbbing pounding pain, and when she is sitting when looking in her peripheral feels it immediately.

3/9/24- Pt reports she does not feel well today.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C4; C6; T1; T5; **C5; T6; T7; L2; L3; L5; Sacrum; SI.**
Tenderness

Left Side
- Cervical Spine (Minimal)

Right Side
- Cervical Spine (Minimal)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)

Right Side
- Cervical Spine (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES
1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)

3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Radiculopathy, cervical region (M54.12)
7. Cervicalgia (M54.2)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
11. Segmental and somatic dysfunction of thoracic region (M99.02)
12. Segmental and somatic dysfunction of upper extremity (M99.07)
13. Strain of muscle(s) and tendon(s) of the rotator cuff of left shoulder, initial encounter (S46.012A)
14. Unspecified superficial injury of left hand, initial encounter (S60.922A)
15. Pain in joints of left hand (M25.542)
16. Other muscle spasm (M62.838)
17. Sleep deprivation (Z72.820)

PLAN

Based on subjective and objective data, today's treatment consists of:

- **CMT 3-4 Regions (98941) consisting of activator technique, diversified technique was provided specifically to C4; C5; C6; T1; T5; T6; T7; L2; L3; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.**
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, left back, left shoulder, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to left neck, right neck, left back, right back, left shoulder, left hand, to reduce pain reported at initial encounter by 25%.
- **Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.**
- **Therapeutic Exercise (Ea. 15 Min) (97110) consisting of lumbar active therapeutic movements was provided specifically to Neck, Back for 10 minutes (1 unit), to reduce pain reported at initial encounter by 25%.**
- **Extraspinal CMT (98943) consisting of diversified technique, to reduce pain reported at initial encounter by 25%.**

130/67 pulse 68.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marcia Smith, DC
Date: 03/14/2024 10:02 AM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Raquel Merriman
Date of Birth: 12/6/1994

Provider: Meka Kennedy, DC
Date of Service: 3/16/2024

SUBJECTIVE

PRIMARY COMPLAINT
Left neck, Right neck

- Aching, Throbbing pain: 8/10
  Radiates to (posterior) left shoulder, left arm, left elbow, left wrist, left hand and left finger. Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Pain with motion: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Stiffness: 3/10
  Described as intermittent (up to 50% of day).

SECOND COMPLAINT
Left shoulder

- Stabbing, Sharp, Aching pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.
- Pain with motion: 9/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.

THIRD COMPLAINT
Left finger, Left hand, Left wrist

- Aching, Sharp pain: 9/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by talking on phone, working, driving, changing positions and ADL's.

- Pain with motion: 9/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by talking on phone, working, driving, changing positions and ADL's.

FOURTH COMPLAINT
Left upper back, Right upper back

- Pain: 7/10
  Described as intermittent (up to 50% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, working, driving, changing positions and ADL's.

FIFTH COMPLAINT
Head

- Aching, Throbbing pain: **From 7 to 9/10 (20% increase)**
  Radiates to (posterior) head. Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by sleeping and ADL's.

Patient states she is having left arm pain that is shooting down her neck and into her shoulder and jaw. Pt also reports chest pain for the last few days. Blood pressure is stable. Headaches have not subsided.
Reports hand weakness is getting worse and grip strength is decreasing.

3/8/24- Pt reports she feels on the left side of her neck throbbing pounding pain, and when she is sitting when looking in her peripheral feels it immediately.

3/9/24- Pt reports she does not feel well today.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C4; C5; C6; T1; T5; T6; T7; L2; L3; L5; Sacrum; SI**.**

Tenderness

Left Side
- Cervical Spine (Minimal)

Right Side
- Cervical Spine (Minimal)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)

Right Side
- Cervical Spine (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)

3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Radiculopathy, cervical region (M54.12)
7. Cervicalgia (M54.2)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
11. Segmental and somatic dysfunction of thoracic region (M99.02)
12. Segmental and somatic dysfunction of upper extremity (M99.07)
13. Strain of muscle(s) and tendon(s) of the rotator cuff of left shoulder, initial encounter (S46.012A)
14. Unspecified superficial injury of left hand, initial encounter (S60.922A)
15. Pain in joints of left hand (M25.542)
16. Other muscle spasm (M62.838)
17. Sleep deprivation (Z72.820)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, diversified technique was provided specifically to C4; C5; C6; T1; T5; T6; T7; L2; L3; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, left back, left shoulder, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to left neck, right neck, left back, right back, left shoulder, left hand, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Therapeutic Exercise (Ea. 15 Min) (97110) consisting of lumbar active therapeutic movements was provided specifically to Neck, Back for 10 minutes (1 unit), to reduce pain reported at initial encounter by 25%.
- **Massage (Ea 15 Min) (97124) was provided specifically to Neck, Right Scapula, Left Scapula, Left Rib Cage, Right Rib Cage, Back for 10 minutes (0 units), to reduce pain reported at initial encounter by 25%.**

Patient received care without incident.

(Electronic Signature)
Provider Name: Meka Kennedy, DC
Date: 03/16/2024 10:07 AM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Raquel Merriman
Date of Birth: 12/6/1994

Provider: Marcia Smith, DC
Date of Service: 3/21/2024

SUBJECTIVE

PRIMARY COMPLAINT
Left neck, Right neck

- Aching, Throbbing pain: **From 8 to 6/10 (20% decrease)**
  Radiates to (posterior) left shoulder, left arm, left elbow, left wrist, left hand and left finger. Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Pain with motion: **From 8 to 6/10 (20% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Stiffness: **From 3 to 5/10 (20% increase)**
  Described as intermittent (up to 50% of day).

SECOND COMPLAINT
Left shoulder

- Stabbing, Sharp, Aching pain: **From 8 to 7/10 (10% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.
- Pain with motion: **From 9 to 7/10 (20% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.

THIRD COMPLAINT
Left finger, Left hand, Left wrist

- Aching, Sharp pain: 9/10
  Described as frequent (up to 75% of day) of **severe intensity (symptom is not alleviated by any amount of modification to ADL)**; increased by talking on phone, working, driving, changing positions and ADL's.

- Pain with motion: 9/10
  Described as frequent (up to 75% of day) of **severe intensity (symptom is not alleviated by any amount of modification to ADL)**; increased by talking on phone, working, driving, changing positions and ADL's.

FOURTH COMPLAINT
Left upper back, Right upper back

- Pain: 7/10
  Described as intermittent (up to 50% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping, sitting, working, driving, changing positions and ADL's.

FIFTH COMPLAINT
Head

- Aching, Throbbing pain: 9/10
  Radiates to (posterior) head. Described as **intermittent to frequent (up to between 50% and 75% of day)** of **severe intensity (symptom is not alleviated by any amount of modification to ADL)**; increased by sleeping and ADL's.

Pt says headaches are getting stronger and now on both sides of head now.

Patient states she is having left arm pain that is shooting down her neck and into her shoulder and jaw. Pt also reports chest pain for the last few days. Blood pressure is stable. Headaches have not subsided.
Reports hand weakness is getting worse and grip strength is decreasing.

3/8/24- Pt reports she feels on the left side of her neck throbbing pounding pain, and when she is sitting when looking in her peripheral feels it immediately.

3/9/24- Pt reports she does not feel well today.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C4; C5; C6; T1; T5; T6; T7; L2; L3; L5; Sacrum; SI**.**

Tenderness

Left Side
- Cervical Spine (Minimal)

Right Side
- Cervical Spine (Minimal)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)

Right Side
- Cervical Spine (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Radiculopathy, cervical region (M54.12)
7. Cervicalgia (M54.2)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
11. Segmental and somatic dysfunction of thoracic region (M99.02)
12. Segmental and somatic dysfunction of upper extremity (M99.07)
13. Strain of muscle(s) and tendon(s) of the rotator cuff of left shoulder, initial encounter (S46.012A)
14. Unspecified superficial injury of left hand, initial encounter (S60.922A)
15. Pain in joints of left hand (M25.542)
16. Other muscle spasm (M62.838)
17. Sleep deprivation (Z72.820)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, diversified technique was provided specifically to C4; C5; C6; T1; T5; T6; T7; L2; L3; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, left back, left shoulder, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to left neck, right neck, left back, right back, left shoulder, left hand, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Therapeutic Exercise (Ea. 15 Min) (97110) consisting of lumbar active therapeutic movements was provided specifically to Neck, Back for 10 minutes (1 unit), to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Marcia Smith, DC
Date: 03/21/2024 03:47 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Raquel Merriman
Date of Birth: 12/6/1994

Provider: Angela Alvarez, DC
Date of Service: 3/22/2024

SUBJECTIVE

PRIMARY COMPLAINT
Left neck, Right neck

- Aching, Throbbing pain: 6/10
  Radiates to (posterior) left shoulder, left arm, left elbow, left wrist, left hand and left finger. Described as frequent to constant (up to between 75% and 100% of day) of **moderate intensity (requires significant or complete modification to ADL)**; increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Pain with motion: 6/10
  Described as frequent to constant (up to between 75% and 100% of day) of **moderate intensity (requires significant or complete modification to ADL)**; increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Stiffness: 5/10
  Described as intermittent (up to 50% of day).

SECOND COMPLAINT
Left shoulder

- Stabbing, Sharp, Aching pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.
- Pain with motion: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.

THIRD COMPLAINT
Left finger, Left hand, Left wrist

- Aching, Sharp pain: 9/10
  Described as frequent (up to 75% of day) of **moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL)**; increased by talking on phone, working, driving, changing positions and ADL's.
- Pain with motion: 9/10
  Described as frequent (up to 75% of day) of **moderate to severe intensity (symptom is**

not alleviated by any amount of modification to ADL); increased by talking on phone, working, driving, changing positions and ADL's.

FOURTH COMPLAINT
Left upper back, Right upper back

- Pain: 7/10
  Described as intermittent (up to 50% of day) of **moderate intensity (requires significant or complete modification to ADL)**; increased by sleeping, sitting, working, driving, changing positions and ADL's.

FIFTH COMPLAINT
Head

- Aching, Throbbing pain: 9/10
  Radiates to (posterior) head. Described as intermittent to frequent (up to between 50% and 75% of day) of severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping and ADL's.

Pt says headaches are getting stronger and now on both sides of head now.

Patient states she is having left arm pain that is shooting down her neck and into her shoulder and jaw. Pt also reports chest pain for the last few days. Blood pressure is stable. Headaches have not subsided.
Reports hand weakness is getting worse and grip strength is decreasing.

3/8/24- Pt reports she feels on the left side of her neck throbbing pounding pain, and when she is sitting when looking in her peripheral feels it immediately.

3/9/24- Pt reports she does not feel well today.

3/22/24- Pt reports she is getting random headaches that are coming in hard and feels like electrical shock in her head. **Pt reports she is able to only get 5-6 hours of sleep**. **Can't sleep on either side because is feeling numbness in her arms**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C5; C6; T1; T6; T7; L2; L3; L5; Sacrum; SI**. No findings: C4; T5**.

Tenderness

Left Side

- Cervical Spine (Minimal)

Right Side

- Cervical Spine (Minimal)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
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8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
11. Segmental and somatic dysfunction of thoracic region (M99.02)
12. Segmental and somatic dysfunction of upper extremity (M99.07)
13. Strain of muscle(s) and tendon(s) of the rotator cuff of left shoulder, initial encounter (S46.012A)
14. Unspecified superficial injury of left hand, initial encounter (S60.922A)
15. Pain in joints of left hand (M25.542)
16. Other muscle spasm (M62.838)
17. Sleep deprivation (Z72.820)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, diversified technique was provided specifically to C5; C6; T1; T6; T7; L2; L3; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, left back, left shoulder, to reduce pain reported at initial encounter by 25%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 25%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to left neck, right neck, left back, right back, left shoulder, left hand, to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Angela Alvarez, DC
Date: 03/22/2024 05:03 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Raquel Merriman
Date of Birth: 12/6/1994

Provider: Marcia Smith, DC
Date of Service: 3/28/2024

SUBJECTIVE

PRIMARY COMPLAINT
Left neck, Right neck

- Aching, Throbbing pain: 6/10
  Radiates to (posterior) left shoulder, left arm, left elbow, left wrist, left hand and left finger. Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Pain with motion: 6/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Stiffness: 5/10
  Described as intermittent (up to 50% of day).

SECOND COMPLAINT
Left shoulder

- Stabbing, Sharp, Aching pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.
- Pain with motion: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.

THIRD COMPLAINT
Left finger, Left hand, Left wrist

- Aching, Sharp pain: 9/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by talking on phone, working, driving, changing positions and ADL's.
- Pain with motion: 9/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is

not alleviated by any amount of modification to ADL); increased by talking on phone, working, driving, changing positions and ADL's.

FOURTH COMPLAINT
Left upper back, Right upper back

- Pain: 7/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by sleeping, sitting, working, driving, changing positions and ADL's.

FIFTH COMPLAINT
Head

- Aching, Throbbing pain: 9/10
  Radiates to (posterior) head. Described as intermittent to frequent (up to between 50% and 75% of day) of severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping and ADL's.

Shoulder pain and arm pain continues. **Patient reports it is throbbing and feels as though care is only working temporarily**. **Patient reports sleep is being affected due to her pain levels.**

Pt says headaches are getting stronger and now on both sides of head now.

Patient states she is having left arm pain that is shooting down her neck and into her shoulder and jaw. Pt also reports chest pain for the last few days. Blood pressure is stable. Headaches have not subsided.
Reports hand weakness is getting worse and grip strength is decreasing.

3/8/24- Pt reports she feels on the left side of her neck throbbing pounding pain, and when she is sitting when looking in her peripheral feels it immediately.

3/9/24- Pt reports she does not feel well today.

3/22/24- Pt reports she is getting random headaches that are coming in hard and feels like electrical shock in her head. Pt reports she is able to only get 5-6 hours of sleep. Can't sleep on either side because is feeling numbness in her arms.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C5; C6; T1; T6; T7; L2; L3; L5; Sacrum; SI**.**

Tenderness

Left Side

- Cervical Spine (Minimal)

Right Side

- Cervical Spine (Minimal)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)
3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Radiculopathy, cervical region (M54.12)
7. Cervicalgia (M54.2)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
11. Segmental and somatic dysfunction of thoracic region (M99.02)
12. Segmental and somatic dysfunction of upper extremity (M99.07)
13. Strain of muscle(s) and tendon(s) of the rotator cuff of left shoulder, initial encounter (S46.012A)
14. Unspecified superficial injury of left hand, initial encounter (S60.922A)
15. Pain in joints of left hand (M25.542)
16. Other muscle spasm (M62.838)
17. Sleep deprivation (Z72.820)

Herniations c2-3 and C3-4. Tear of left TFC in wrist.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of activator technique, diversified technique was provided specifically to C5; C6; T1; T6; T7; L2; L3; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
- **Report of Findings (99212) was provided specifically to , to enable patient to manage injury without re-aggravation.**

Reviewed MRI results referred to ortho ASAP

Patient received care without incident.

(Electronic Signature)
Provider Name: Marcia Smith, DC
Date: 03/28/2024 10:06 AM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Raquel Merriman
Date of Birth: 12/6/1994

Provider: Meka Kennedy, DC
Date of Service: 4/4/2024

SUBJECTIVE

PRIMARY COMPLAINT
Left neck, Right neck

- Aching, Throbbing pain: 6/10
  Radiates to (posterior) left shoulder, left arm, left elbow, left wrist, left hand and left finger. Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Pain with motion: 6/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate intensity (requires significant or complete modification to ADL); increased by sleeping, sitting, talking on phone, working, driving, changing positions and ADL's.
- Stiffness: 5/10
  Described as intermittent (up to 50% of day).

SECOND COMPLAINT
Left shoulder

- Stabbing, Sharp, Aching pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.
- Pain with motion: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by lifting, working, driving, changing positions and ADL's.

THIRD COMPLAINT
Left finger, Left hand, Left wrist

- Aching, Sharp pain: 9/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by talking on phone, working, driving, changing positions and ADL's.
- Pain with motion: 9/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by talking on phone, working, driving, changing positions and ADL's.

FOURTH COMPLAINT
Left upper back, Right upper back

- Pain: 7/10
  Described as intermittent (up to 50% of day) of moderate intensity (requires significant or complete modification to ADL); increased by sleeping, sitting, working, driving, changing positions and ADL's.

FIFTH COMPLAINT
Head

- Aching, Throbbing pain: 9/10
  Radiates to (posterior) head. Described as intermittent to frequent (up to between 50% and 75% of day) of severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by sleeping and ADL's.

Shoulder pain and arm pain continues. Patient reports it is throbbing and feels as though care is only working temporarily. Patient reports sleep is being affected due to her pain levels.

Pt says headaches are getting stronger and now on both sides of head now.

Patient states she is having left arm pain that is shooting down her neck and into her shoulder and jaw. Pt also reports chest pain for the last few days. Blood pressure is stable. Headaches have not subsided.
Reports hand weakness is getting worse and grip strength is decreasing.

3/8/24- Pt reports she feels on the left side of her neck throbbing pounding pain, and when she is sitting when looking in her peripheral feels it immediately.

3/9/24- Pt reports she does not feel well today.

3/22/24- Pt reports she is getting random headaches that are coming in hard and feels like electrical shock in her head. Pt reports she is able to only get 5-6 hours of sleep. Can't sleep on either side because is feeling numbness in her arms.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C5; C6; T1; T6; T7; L2; L3; L5; Sacrum; SI.

Tenderness

Left Side

- Cervical Spine (Minimal)

Right Side

- Cervical Spine (Minimal)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Shoulder (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
2. Acute post-traumatic headache, intractable (G44.311)

3. Disorder of ligament, vertebrae (M24.28)
4. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Radiculopathy, cervical region (M54.12)
7. Cervicalgia (M54.2)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
11. Segmental and somatic dysfunction of thoracic region (M99.02)
12. Segmental and somatic dysfunction of upper extremity (M99.07)
13. Strain of muscle(s) and tendon(s) of the rotator cuff of left shoulder, initial encounter (S46.012A)
14. Unspecified superficial injury of left hand, initial encounter (S60.922A)
15. Pain in joints of left hand (M25.542)
16. Other muscle spasm (M62.838)
17. Sleep deprivation (Z72.820)

Herniations c2-3 and C3-4. Tear of left TFC in wrist.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of **Instrument adjusting**, diversified technique was provided specifically to C5; C6; T1; T6; T7; L2; L3; L5; Sacrum; SI, to reduce pain reported at initial encounter by 25%.
- **Extraspinal CMT (98943) consisting of Instrument adjusting, joint mobilization, to reduce pain reported at initial encounter by 25%.**
- **Massage (Ea 15 Min) (97124) was provided specifically to Left Scapula, Right Scapula, Neck, Back for 10 minutes (1 unit), to reduce pain reported at initial encounter by 25%.**

Patient received care without incident.

(Electronic Signature)
Provider Name: Meka Kennedy, DC
Date: 04/04/2024 11:30 AM

ORTHO SPORT & SPINE PHYSICIANS

NORTHSIDE RADIOLOGY Associates
5775 Glenridge Drive, NE, Bldg B Ste B525
Atlanta, GA 30328-5380
(404) 252-4709
www.northsideradiology.com

Performing Location: OSSP - Morrow

Patient Name:	MERRIMAN, RAQUEL	**Procedure Date:**	April 9, 2024 11:01
Patient ID:	AB121430OSSP	**Referring Physician:**	1851762702, Sachs
Date of Birth:	December 6, 1994	**Accession Number:**	1939DFUT423550OSSP
Gender:	Female	**Procedure Description:**	MRI Shoulder w/o Contrast Left

MRI LEFT SHOULDER APRIL 9, 2024

HISTORY: Motor vehicle accident injury with left shoulder pain

TECHNIQUE: Routine MRI was performed.

COMPARISON: None

FINDINGS: The AC joint is unremarkable. Acromial morphology is curved. Subcortical cystic changes are present in the posterior aspect of the humeral head. There is no evidence of recent bony injury.

Mild fluid in subacromial/subdeltoid bursa is suggestive of mild bursitis. Mild distal supraspinatus tendinosis is present. The supraspinatus, infraspinatus, teres minor and subscapularis tendons are intact. Muscle volume is normal.

The labrum and long head of the biceps tendon are intact. No focal glenohumeral chondral defects or loose bodies are present.

IMPRESSION: Mild subacromial/subdeltoid bursitis. Mild distal supraspinatus tendinosis.

Thank you for your referral. For physician to physician consultation about this report or any other Radiology matter, you can reach me or any of our Radiologists through Northside Radiology Associates Customer Care Line at 404-649-6600.

Electronically Signed by Patrick Datoc MD on 4/11/2024 5:12 AM

LOCATION: NRAWRS04LG7

MERRIMAN, Raquel **DOB:** 12/06/1994 (29 yo F) **Acc No.** AB121430 **DOS:** 04/02/2024

Patient: MERRIMAN, Raquel
Account Number: AB121430
DOB: 12/06/1994 **Age:** 29 Y **Sex:** Female
Phone: 404-801-7060
Address: 80 DERBY COUNTRY DR, ELLENWOOD, GA-30294-2731

Provider: Tamara Sachs, PA-C
Date: 04/02/2024

Subjective:

Chief Complaints:

1. Patient presents today complaining of neck, left shoulder and left wrist pain..

HPI:

Constitutional:

29-year-old female comes in today after motor vehicle accident on February 13, 2024. She was restrained driver, airbags did not deploy. She was involved in a sideswipe type collision she denies any head injury or loss of consciousness associated with the accident. After the accident she did go to the emergency room where they did x-rays of her wrist and she was discharged home the same day. She has subsequently been doing chiropractic care which has been minimally helpful. She has had MRI imaging of her neck and her wrist which she brings today for review. Since the accident her primary complaint has been neck, left shoulder pain, left wrist pain.

Neck pain is a 0 out of 10. She does complain of some left upper extremity paresthesias with numbness and tingling mostly into the shoulder and upper arm. Does report some weakness into her left arm and hand primarily her wrist. Denies clumsiness in the hands. Does report some balance issues.

Left wrist pain is rated as a 10 out of 10 mostly on the radial side of the wrist. Worsening with using her hands in her job as a hairdresser as well as an Amazon worker, Heavy lifting, she does complain's of weakness into her wrist and hand.

Left shoulder pain is a 9 out of 10 described as constant achiness into her shoulder worsening with overhead activities and heavy lifting as well as sleeping on that side at night.

ROS:

General/Constitutional:

Unexplained weight loss denies, denies. Night sweats denies, denies. Marked fatigue denies, denies. Difficulty sleeping denies, denies.

Ophthalmologic:

Glasses denies, denies. Change of vision denies, denies.

ENT:

Decreased hearing denies, denies. Difficulty swallowing denies, denies. Ear pain denies, denies. Nosebleed denies, denies. Hoarseness denies, denies.

Endocrine:

Admits Weakness.

Respiratory:

Productive cough or sputum denies, denies. Morning cough denies, denies. Shortness of breath denies, denies.

Gastrointestinal:

Stomach pain / ulcers denies, denies. Blood in stool denies, denies. Constipation denies, denies. Diarrhea denies, denies. Hemorrhoids denies

, denies

. Nausea denies, denies. Vomiting denies, denies.

Genitourinary:

Burning with urination denies, denies. Difficulty starting urination denies, denies. Pelvic pain denies, denies. Unable to empty bladder denies, denies.

Musculoskeletal:

Neck pain denies, denies. Back pain denies, denies. Muscle aches denies, denies. Painful joints **admits Left Shoulder and Left Wrist Pain**.

Skin:

Swollen ankles denies, denies. Easy bruising denies, denies. Rash denies, denies.

Neurologic:
Fainting denies, denies. Headache denies, denies. Seizures denies, denies. Tremor denies, denies.

Medical History: Denies All Past Medical History, Asthma, Bronchitis.

Surgical History: C Section X 3 2015, 2017, 2022 , Scar tissue removed from abdomen 2019 , Cyst removed from left ovary 2019 .

Hospitalization/Major Diagnostic Procedure: C Section X 3 .

Family History: Father: deceased. Mother: alive, Diabetes Hypertension Anxiety.

Social History:
Tobacco Use:
Tobacco Use/Smoking
Are you a *Current smoker*
Hand dominance:
Hand Dominance: Right Handed.
Drugs/Alcohol:
Alcohol Screen (Audit-C)
Did you have a drink containing alcohol in the past year? *No*
Points *0*
Interpretation *Negative*
Miscellaneous:
Occupation: Amazon.

Medications: None

Allergies: Rocephan: Hives.

Implants: No known implant history.

Objective:

Vitals: Ht: **5 ft 6 in**, Wt: **171.2** lbs, BMI: **27.63** Index, BP: **sitting:115/75** mm Hg, HR: **70** /min, Pain scale: **9** 1-10.

Examination:
General Examination:
MRI of the left wrist from MRI imaging specialist dated study March 22, 2024 revealed TFCC demonstrates small intrasubstance tear of the TFCC attachment point to the ulnar styloid.
MRI of the cervical spine revealed C3-C4, C2-C3 disc herniations with no spinal cord or neuroforaminal compression.
Examination of the wrist shows full range of motion both actively and passively. Tenderness to palpation over the radial side of the wrist, as well as the radial side. negative Finklestein's, Negative Fovea sign. Grip strength is 4/5 compared to the contralateral side which is 5/5. There is no sensory deficit. Neurovascularly intact distally.
Examination of the left shoulder shows full range of motion both actively and passively. Positive Neer test, Positive Hawkins, Negative cross-chest adduction, negative speeds, negative Jobe, Negative Obriens, negative Yergasons, Negative Lift-off, Negative belly press. Nontender over the coracoid, Nontender over the A/C joint, nontender over the long head of the biceps tendon. Has 5/5/ strength with resisted abduction, and resisted external rotation.

Assessment:

Assessment:

1. Cervical facet syndrome - M47.812
2. Left wrist pain - M25.532
3. Left shoulder pain - M25.512

Plan:

Treatment:

MERRIMAN, Raquel **DOB:** 12/06/1994 (29 yo F) **Acc No.** AB121430 **DOS:** 04/02/2024

1. Cervical facet syndrome
Start Meloxicam Tablet, 15 MG, 1 tablet, Orally, Once a day, 30 day(s), 30, Refills 0 ; Start Dendracin Neurodendraxcin Lotion, 0.025-10-30 %, Apply as needed to affected area, Externally, 2-3 times a day, 30 days, 1 Tube, Refills 0 ; Start Cyclobenzaprine HCl Tablet, 10 MG, 1 tablet as needed, Orally, Once a day at bedtime, 30 days, 30, Refills 0 ; Start TENS Therapy Pain Relief Device, -, as directed, as needed ; Start LidoPro Ointment, 4-.0325-10-27.5 %, 1 application as needed, Externally, Four times a day .

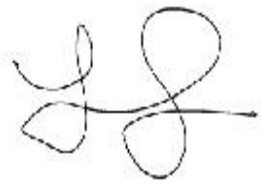
2. Left wrist pain
Start Wrist Brace- Cock Up Brace, as directed .

3. Left shoulder pain
Imaging: MRI Left Shoulder wo (Ordered for 04/09/2024)
Kendrick, Teneeshia 4/2/2024 11:10:31 AM >

4. Others
Notes: 29-year-old female with neck, left shoulder, left wrist pain status post motor vehicle accident on February 13, 2024. I reviewed MRI of the cervical spine which revealed disc herniations C2-C3 as well as C3-C4 with no spinal cord or neuroforaminal compression. She does have some pain radiating into her left shoulder as well as into her left wrist. I have reviewed MRI of the left wrist which revealed small intrasubstance tear of the TFC attachment point to the ulnar styloid. She was given cock up wrist brace. She was given prescription for meloxicam and cyclobenzaprine as well as Dendracin and LidoPro creams. She was referred for MRI of her shoulder.

Dictated by Tamara Sachs, PA-C for Dr. Zahra Merchant, MD

Procedure Codes: 99070 MOBIC/MELOXICAM 15MG #30, 99070 DENDRACIN 30/10/0.25%, 99070 CYCLOBENZAPRINE 10MG #30, 99070 LIDOPRO OINT, L3908 WRIST Cock-up, Modifiers: NU , LT, E0730 Tens four lead, Modifiers: NU , T2002 #1 Non-emergency transportation; per diem



Electronically signed by Tamara Sachs , PA-C on 04/05/2024 at 02:56 PM EDT
Sign off status: Completed
Disposition and Communication:

Provider: Tamara Sachs, PA-C **Date: 04/02/2024**

MERRIMAN, Raquel **DOB:** 12/06/1994 (29 yo F) **Acc No.** AB121430 **DOS:** 04/15/2024

Patient: MERRIMAN, Raquel
Account Number: AB121430
DOB: 12/06/1994 **Age:** 29 Y **Sex:** Female
Phone: 404-801-7060
Address: 80 DERBY COUNTRY DR, ELLENWOOD, GA-30294-2731

Provider: Tamara Sachs, PA-C
Date: 04/15/2024

Subjective:

Chief Complaints:

1. Patient returns complaining of neck, left shoulder and left wrist pain...

HPI:

Constitutional:

29-year-old female comes in today for follow-up visit she is here to follow-up on her neck, left shoulder, left wrist pain status post motor vehicle accident on February 30 13th 2024. kSince her last visit patient has stopped doing chiropractic care as it was aggravating her pain. Patient reports good relief of her neck and left wrist pain with stopping chiropractic. Pain in her neck has been very mild 2-3 out of 10. She denies any bilateral upper extremity paresthesias with numbness or tingling. Left wrist pain has also been very mild has been a 3 out of 10 worsening with activities and heavy lifting. Her primary complaint is her left shoulder pain which is a 5 out of 10 and worsening with when she is being overactive or sleeping.

MRI of the left shoulder from.

ROS:

General/Constitutional:

Weight Gain denies. Unexplained weight loss denies. Change in appetite denies. Fever or chills denies. Night sweats denies. Marked fatigue denies. Difficulty sleeping denies.

Gastrointestinal:

Bowel / Bladder issues denies. Stomach pain / ulcers denies. Blood in stool denies. Constipation denies. Diarrhea denies. Hemorrhoids denies. Nausea denies. Vomiting denies.

Genitourinary:

Abdominal pain/swelling denies. Difficulty urinating denies. Burning with urination denies. Pelvic pain denies. Unable to empty bladder denies.

Musculoskeletal:

Neck pain **admits**. Back pain denies. Muscle aches denies. Painful joints **admits Left Shoulder and Left Wrist Pain**.

Neurologic:

Balance difficulty denies. Fainting denies. Headache denies. Seizures denies. Tingling/Numbness denies. Blurred Vision denies. Tremor denies.

Medical History: Denies All Past Medical History, Asthma, Bronchitis.

Surgical History: C Section X 3 2015, 2017, 2022 , Scar tissue removed from abdomen 2019 , Cyst removed from left ovary 2019 .

Hospitalization/Major Diagnostic Procedure: C Section X 3 .

Family History: Father: deceased. Mother: alive, Diabetes Hypertension Anxiety.

Social History:

Tobacco Use:

Tobacco Use/Smoking

Are you a *Current smoker*

Hand dominance:

Hand Dominance: Right Handed.

Drugs/Alcohol:

Alcohol Screen (Audit-C)

Did you have a drink containing alcohol in the past year? *No*

Points *0*

MERRIMAN, Raquel **DOB:** 12/06/1994 (29 yo F) **Acc No.** AB121430 **DOS:** 04/15/2024

Interpretation *Negative*
Miscellaneous:
Occupation: Amazon.

Medications: Taking Meloxicam 15 MG Tablet 1 tablet Orally Once a day , stop date 05/02/2024, Taking Dendracin Neurodendraxcin 0.025-10-30 % Lotion Apply as needed to affected area Externally 2-3 times a day , stop date 05/02/2024, Taking Cyclobenzaprine HCl 10 MG Tablet 1 tablet as needed Orally Once a day at bedtime , stop date 05/02/2024, Taking TENS Therapy Pain Relief - Device as directed , Taking LidoPro 4-.0325-10-27.5 % Ointment 1 application as needed Externally Four times a day , Taking Wrist Brace- Cock Up Brace as directed , Medication List reviewed and reconciled with the patient

Allergies: Rocephan: Hives.

Implants: No known implant history.

Objective:

Vitals: Ht: **5 ft 6 in**, Wt: **179** lbs, BMI: **28.89** Index, BP: **sitting:104/74** mm Hg, HR: **69** /min, Pain scale: **6** 1-10.

Examination:
General Examination:
Examination of the wrist shows full range of motion both actively and passively. Tenderness to palpation over the radial side of the wrist, as well as the radial side. negative Finklestein's, Negative Fovea sign. Grip strength is 4/5 compared to the contralateral side which is 5/5. There is no sensory deficit. Neurovascularly intact distally.
Examination of the left shoulder shows full range of motion both actively and passively. Positive Neer test, Positive Hawkins, Negative cross-chest adduction, negative speeds, negative Jobe, Negative Obriens, negative Yergasons, Negative Lift-off, Negative belly press. Nontender over the coracoid, Nontender over the A/C joint, nontender over the long head of the biceps tendon. Has 5/5/ strength with resisted abduction, and resisted external rotation.
April 9, 2024 Ortho sport and spine revealed mild subacromial subdeltoid bursitis no rotator cuff tear.

Assessment:

Assessment:

1. Left shoulder pain - M25.512 (Primary)
2. Cervical facet syndrome - M47.812

Plan:

Treatment:
1. Left shoulder pain
Procedure: PRP injection (Ordered for 04/29/2024) Morrow
PRP Injection Left Shoulder x 1 Kendrick, Teneeshia 4/15/2024 11:57:19 AM >
Procedure: PRP injection (Ordered for 06/14/2024) Morrow
X1 PRP Injection Left Shoulder x 1 Kendrick, Teneeshia 4/15/2024 11:56:19 AM >

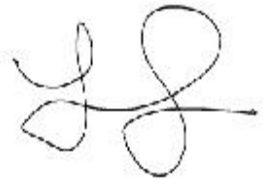
2. Others
Notes: 29-year-old female comes in today for follow-up visit she is here to follow-up on her neck, left shoulder, left wrist pain status post motor vehicle accident on February 30 13th 2024. I reviewed MRI of the left shoulder which revealed mild subacromial subdeltoid bursitis no rotator cuff tear. At this time I recommended PRP injection for the left shoulder x 2 risk benefits and complications were discussed and she elected to proceed.. For the neck and the left wrist she will continue conservative care given her good relief of her pain with conservative treatment.

Dictated by Tamara Sachs, PA-C for Dr. Zahra Merchant, MD

Procedure Codes: T2002 #1 Non-emergency transportation; per diem

Follow Up: 3 Weeks

MERRIMAN, Raquel **DOB:** 12/06/1994 (29 yo F) **Acc No.** AB121430 **DOS:** 04/15/2024



Electronically signed by Tamara Sachs , PA-C on 04/22/2024 at 04:29 PM EDT
Sign off status: Completed
Disposition and Communication:

Provider: Tamara Sachs, PA-C **Date: 04/15/2024**

MERRIMAN, Raquel **DOB:** 12/06/1994 (29 yo F) **Acc No.** AB121430 **DOS:** 04/26/2024

MERRIMAN, Raquel
29 Y old Female, DOB: 12/06/1994
80 DERBY COUNTRY DR, ELLENWOOD, GA-30294-2731
Home: 404-801-7060
Surgeon: Zahra Merchant, MD

04/26/2024 **Zahra Merchant, MD**

OPERATIVE REPORT

Anesthesia:

Indications:

Details of Procedure:

Specimens:

Complications:

Findings:

Notes:

LEFT SHOULDER PRP INJECTION

INDICATIONS: Please see last progress note. Patient agrees to LEFT shoulder PRP injection.

PRE PROCEDURE DIAGNOSIS: LT Subacromial Bursitis; LT Rotator Cuff Tendonitis/Tendinopathy

POST PROCEDURE DIAGNOSIS: SAME.

PROCEDURE: LEFT SHOULDER PRP INJECTION UNDER FLUOROSCOPIC GUIDANCE

SEDATION: None

PROCEDURE NARRATIVE:
After informed consent was obtained, blood was drawn/collected and spun down, plasma was collected and placed in syringe.

The patient was brought to the procedure suite and placed in a prone position on the exam table. The posterior left shoulder was prepped and draped in the usual sterile fashion. Then, using fluoroscopic guidance to improve accuracy, efficacy, and decrease pain, a 1.5inch 25gauge needle was atraumatically introduced and advanced under fluoroscopic guidance into the posterior aspect of the subacromial space under direct fluoroscopic visualization.

After confirmation of the position of the needle tip 5cc's of platelet rich plasma was injected into the region. The patient tolerated the procedure well without immediate complication.

Patient was monitored for an appropriate length of time before being discharged to home. Vital signs remained stable throughout the procedure and in recovery. There were no complications.

Zahra Merchant, M.D.

Procedure Codes:

MERRIMAN, Raquel **DOB:** 12/06/1994 (29 yo F) **Acc No.** AB121430 **DOS:** 04/26/2024

1.A4649 PRP ANGEL KIT.
2.0232T EXTREMITY NJX PLATELET PLASMA.
3.99070 LIDOPRO PATCH.

Electronically signed by Zahra Merchant , MD on 04/26/2024 at 05:57 PM EDT
Sign off status: Completed

OSSP CONYERS
1292 WELLBROOK CIR NE
CONYERS, GA 30012-8031
Tel: 678-752-7246
Fax:

Progress Note: Zahra Merchant, MD 04/26/2024

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

MERRIMAN, Raquel **DOB:** 12/06/1994 (29 yo F) **Acc No.** AB121430 **DOS:** 05/03/2024

MERRIMAN, Raquel
29 Y old Female, DOB: 12/06/1994
80 DERBY COUNTRY DR, ELLENWOOD, GA-30294-2731
Home: 404-801-7060
Surgeon: Roderica E Cottrell, MD

05/03/2024 Roderica E Cottrell, MD

OPERATIVE REPORT

Pre-op. Diagnosis:
1.Left subacromial bursitis
2.Left rotator cuff tendinopathy

Post-op. Diagnosis:
1.Same as pre-op diagnosis

Operation:
1.Plasma rich protein injection
2.Left subacromial space
3.Fluoroscopic guidance

Anesthesia:
Local

Indications:
Severe pain unresponsive to non-invasive measures

Details of Procedure:
Platelet-Rich Plasma Injection

The details of the procedure were reviewed, and informed consent was obtained. 60mL of patient's blood was obtained under sterile technique and 6 mL of platelet-rich plasma was obtained following the appropriate protocol. The patient was taken to the fluoroscopy suite and placed on the table in the prone position. Procedure site, side, and levels were verified with time-out. Patient's skin was cleaned with surgical prep and was draped in the usual sterile fashion. Patient's skin and local tissues were infiltrated with 1% lidocaine and sodium bicarbonate to reduce burning effect using a 25-gauge needle. Next, a 22-gauge 3.5-inch needle was inserted under fluoroscopic guidance until the tip was visualized in the intra-artiuclar shoulder joint space. Arthrogram confirmation was made follwing the injection of 0.5 cc of Omnipaque 300. Following negative aspiration to confirm no vascular uptake, 6 mL of platelet rich plasma was injected without complications. The needle was withdrawn, and a band-aid was applied to the injection site.

The patient was taken to the recovery area and monitored for an appropriate length of time before being discharged to home. Vital signs remained stable throughout the procedure and in recovery. There were no complications.

Specimens:
none

Complications:
none

Findings:
as above

MERRIMAN, Raquel **DOB:** 12/06/1994 (29 yo F) **Acc No.** AB121430 **DOS:** 05/03/2024

Notes:

Procedure Codes:
1.A4649 PRP ANGEL KIT.
2.0232T EXTREMITY NJX PLATELET PLASMA.

Electronically signed by Roderica Cottrell , MD on 05/06/2024 at 03:41 PM EDT
Sign off status: Completed

OSSP CONYERS
1292 WELLBROOK CIR NE
CONYERS, GA 30012-8031
Tel: 678-752-7246
Fax:

Progress Note: Roderica E Cottrell, MD 05/03/2024

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

Patient: MERRIMAN, Raquel
Account Number: AB121430
DOB: 12/06/1994 **Age:** 29 Y **Sex:** Female
Phone: 404-801-7060
Address: 80 DERBY COUNTRY DR, ELLENWOOD, GA-30294-2731

Provider: Tamara Sachs, PA-C
Date: 05/17/2024

Subjective:

Chief Complaints:

1. Patient returns complaining of neck, left shoulder and left wrist pain..

HPI:

Constitutional:

29-year-old female comes in today for follow-up visit she is here to follow-up on her neck, left shoulder, left wrist pain status post motor vehicle accident February 13, 2024. Since her last visit she has undergone PRP injection to her left shoulder April 26 and May 3 x 2. At today's visit she reports no relief after undergoing this injection. States that she has increasing neck pain and shoulder pain after undergoing this procedure she did not even get 1 day of relief. Neck pain is a 7 out of 10 described as constant achiness into the cervical paraspinal musculature bilateral scapular region worsening with heavy lifting in her job. She denies any bilateral upper extremity paresthesias with numbness or tingling.

ROS:

General/Constitutional:

Weight Gain denies. Unexplained weight loss denies. Change in appetite denies. Fever or chills denies. Night sweats denies. Marked fatigue denies. Difficulty sleeping denies.

Gastrointestinal:

Bowel / Bladder issues denies. Stomach pain / ulcers denies. Blood in stool denies. Constipation denies. Diarrhea denies. Hemorrhoids denies. Nausea denies. Vomiting denies.

Genitourinary:

Abdominal pain/swelling denies. Difficulty urinating denies. Burning with urination denies. Pelvic pain denies. Unable to empty bladder denies.

Musculoskeletal:

Neck pain **admits**. Back pain denies. Muscle aches denies. Painful joints **admits right shoulder,right wrist**.

Neurologic:

Balance difficulty denies. Fainting denies. Headache denies. Seizures denies. Tingling/Numbness denies. Blurred Vision denies. Tremor denies.

Medical History: Denies All Past Medical History, Asthma, Bronchitis.

Surgical History: C Section X 3 2015, 2017, 2022 , Scar tissue removed from abdomen 2019 , Cyst removed from left ovary 2019 .

Hospitalization/Major Diagnostic Procedure: C Section X 3 .

Family History: Father: deceased. Mother: alive, Diabetes Hypertension Anxiety.

Social History:

Tobacco Use:

Tobacco Use/Smoking

Are you a *Current smoker*

Hand dominance:

Hand Dominance: Right Handed.

Drugs/Alcohol:

Alcohol Screen (Audit-C)

Did you have a drink containing alcohol in the past year? *No*

MERRIMAN, Raquel **DOB:** 12/06/1994 (29 yo F) **Acc No.** AB121430 **DOS:** 05/17/2024

Points *0*
Interpretation *Negative*

Medications: Taking LidoPro 4-4-5 % Patch 1 patch as needed Externally Twice a day , Unknown TENS Therapy Pain Relief - Device as directed , Unknown LidoPro 4-.0325-10-27.5 % Ointment 1 application as needed Externally Four times a day , Unknown Wrist Brace- Cock Up Brace as directed , Medication List reviewed and reconciled with the patient

Allergies: Rocephan: Hives.

Implants: No known implant history.

Objective:

Vitals: Ht: **5 ft 6 in**, Wt: **181.6** lbs, BMI: **29.31** Index, BP: **sitting:109/70** mm Hg, HR: **67** /min.

Examination:

General Examination:

Examination of the wrist shows full range of motion both actively and passively. Tenderness to palpation over the radial side of the wrist, as well as the radial side. negative Finklestein's, Negative Fovea sign. Grip strength is 4/5 compared to the contralateral side which is 5/5. There is no sensory deficit. Neurovascularly intact distally.

Examination of the left shoulder shows full range of motion both actively and passively. Positive Neer test, Positive Hawkins, Negative cross-chest adduction, negative speeds, negative Jobe, Negative Obriens, negative Yergasons, Negative Lift-off, Negative belly press. Nontender over the coracoid, Nontender over the A/C joint, nontender over the long head of the biceps tendon. Has 5/5/ strength with resisted abduction, and resisted external rotation.

Assessment:

Assessment:

1. Cervical facet syndrome - M47.812

Plan:

Treatment:

1. Cervical facet syndrome

Start Dendracin Neurodendraxcin Lotion, 0.025-10-30 %, Apply as needed to affected area, Externally, 2-3 times a day, 30 days, 1 Tube, Refills 0 ; Start LidoPro Ointment, 4-.0325-10-27.5 %, 1 application as needed, Externally, Four times a day .

Procedure: Cervical IAF (Bilateral C3-C6) (Ordered for 07/16/2024) Morrow
Cervical IAF Bilateral C3-C6 Christina, Sams 5/17/2024 12:07:54 PM >

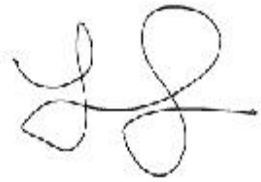
Referral To:PT Open Referral Physical Therapist
Reason:PT Treatment 2-3x a week for 6 weeks for Cervical Facet syndrome in Ellenwood area

2. Others

Notes: 29-year-old female comes in today for follow-up visit she is here to follow-up on her neck, left shoulder, left wrist pain status post motor vehicle accident February 13, 2024. Since her last visit she has undergone PRP injection to her left shoulder April 26 and May 3 x 2. At today's visit she reports no relief after undergoing this injection. States that she has increasing neck pain and shoulder pain after undergoing this procedure she did not even get 1 day of relief. Neck pain is a 7 out of 10 I have discussed treatment options for the neck to include continued conservative treatment as well as potential bilateral cervical intra-articular facet injections. Risk benefits and complications were discussed she elected to proceed. She was also given a referral for physical therapy
Dictated by Tamara Sachs, PA-C for Dr. Zahra Merchant, MD

Procedure Codes: 99070 DENDRACIN 30/10/0.25%, 99070 Lidothol Gel Lidocaine 4.5%/Menthol

MERRIMAN, Raquel **DOB:** 12/06/1994 (29 yo F) **Acc No.** AB121430 **DOS:** 05/17/2024



Electronically signed by Tamara Sachs , PA-C on 05/23/2024 at 09:53 AM EDT
Sign off status: Completed
Disposition and Communication:

Provider: Tamara Sachs, PA-C **Date: 05/17/2024**